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A longitudinal analysis of person-centred therapy with suicidal clients

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Abstract

Background: There have been substantial research efforts demonstrating the effectiveness of person-centred therapy. However, little research has investigated whether person-centred therapy is effective in facilitating psychological growth amongst clients experiencing suicidal ideation and serious mental health difficulties.

Aim: This study aimed to determine whether suicidal clients who received person-centred therapy experienced increased levels of authenticity, well-being and psychological distress. The predictive validity of authenticity and well-being upon psychological distress was also tested.

Method: The study utilised quantitative, longitudinal methodology. Data were collected from a clinical sample of clients receiving person-centred therapy at a counselling research clinic ($N = 56$) over the course of 20 sessions.

Results: There were statistically significant improvements in levels of authenticity, well-being and psychological distress over 20 sessions of therapy; a minimum of 15 sessions were required for significant change to be observed. Authenticity and well-being were negatively associated with psychological distress, whilst authenticity and well-being were positively associated with each other. Early authenticity and well-being predict levels of distress later in therapy. These results provide initial evidence to support Rogers' theory of therapy, which is suitable for clients experiencing both mild and severe distress; the findings refute the view that person-centred therapy is only suitable for the "worried well."

Implications: There is now preliminary justification for person-centred therapy being suitable for suicidal clients. Person-centred therapists could consider offering suicidal clients at least 15 sessions to achieve meaningful change; ethical considerations pertaining to this are explored.

KEYWORDS
authenticity, psychological distress, suicidal ideation, therapy outcome, well-being

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1 | INTRODUCTION

The psychological and emotional well-being of the general population of the United Kingdom has been a rising public concern. There has been particular concern about the increasing rates of suicide. Reports from The Office for National Statistics (2019) revealed there were 6507 registered deaths by suicide in 2018; this is an 11.8% rise since 2017. In the last quarter of 2019, rates of suicide within the United Kingdom reached a 19-year high, with 11.4 registered deaths by suicide per 100,000 people (ibid.). Developing psychotherapies suitable for helping people who experience suicidal ideation is of immediate concern within the profession.

There has been a surge in demand for the identification of, and increasing access to, psychotherapies that are effective for working with suicidal clients as they are considered a particularly vulnerable sample within the population (Department for Health and Social Care, 2019; House of Commons, 2019; Public Health England, 2019). There has also been an increase in empirical research investigating and demonstrating the effectiveness of a range of therapeutic approaches to working with people experiencing suicidal ideation. One of the largest reviews of suicide intervention was conducted by Zalsman et al. (2016) and included 1797 studies, 23 systematic reviews, 12 meta-analyses, 67 cohort trials, 40 randomised controlled trials and 22 population-based investigations. The review found the most effective psychological therapies for working with individuals experiencing suicidal ideation were as follows: cognitive behavioural therapy (CBT), dialectical behavioural therapy (DBT), community-based intervention, group therapy, family therapy and multisystemic therapy. Furthermore, meta-analytic research also points to the efficacy of the following: existential therapy and rational emotive behavioural therapy (Cuijpers et al., 2013); interpersonal therapy, psychoanalytic therapy, and outpatient care (D'Anci & Uhl, 2020; Méndez-Bustos et al., 2019); behavioural activation (Lejuez et al., 2011); mentalisation-based therapy (Ougrin et al., 2015); mindfulness-based therapy (Barnhofer et al., 2015); online CBT (Büscher et al., 2020; Hedman et al., 2012; Leavey & Hawkins, 2017; van Straten et al., 2014); and self-help intervention (Torok et al., 2020). No research has investigated the effects of person-centred therapy on people experiencing suicidal ideation.

Previous research on psychotherapy with suicidal clients has largely sought to measure its effectiveness using the level of suicidal ideation as the primary outcome. This reveals how, within mainstream psychotherapeutic thinking, most approaches to psychotherapy take a moral stance towards suicide (and suicidal ideation), presuming that the client is universally in a state of disorder of which their suicidal ideation is evidence. This stance is much the same as that taken by the broader psychotherapy specificity myth proposed by Bozarth (2002) and that demands specific treatments are required for specific problems, such that successful psychotherapy equates with remediating specific symptoms of disorders. Person-centred theory, on the contrary, presents a challenging alternative to, and nonpathologising perspective on, mainstream therapies for psychological distress. Person-centred therapy does not target specific symptoms or adapt its process in the light of psycho diagnosis or formulation; instead, it adopts a holistic approach towards clients. In person-centred theory, psychological distress can be understood as the tension experienced as a result of incongruence between the self-concept and experience. The cause of this incongruence is the development of conditions of worth that might arise through conditional parenting, educational experiences, peer groups, social factors such as classism, racism, ableism, ageism and other cultural sources of external control, such as religion.

In some instances, according to person-centred theory, suicidal ideation may be the result of thwarted potential of the actualising tendency that results in the denial or distortion of experience to cause psychological tension. However, as person-centred theory does not claim to know more about a client than a client knows about themselves, there might be other unique and personal reasons leading to suicidal ideation. For example, the theory also holds that it is entirely possible for a client to experience their life as becoming unliveable when their suffering is too great and that is not the result of distorted experience (Murphy et al., 2022). In this instance, suicidal ideation is not a distortion of reality, but is a congruent experiencing of their suffering. It is not difficult to imagine a person who is suffering from an incurable degenerative condition who decides, quite congruently, they do not wish to live through the process of degeneration and thus experiences suicidal ideation or, for that matter, completes a suicide. The person-centred approach does not take a moral stance towards another person's experiencing nor does it determine for another person when a life is or is not liveable. In a study of person-centred therapy with clients experiencing suicidal ideation, the intention is not to test whether the therapy can remove suicidal ideation as a symptom, but is instead concerned with whether person-centred therapy is associated with an increase in client authenticity and

Implications for Practice and Policy

- In the NHS, person-centred experiential counselling for depression (PCE-CfD) is one of the currently available therapies for people experiencing depression. Depression is not only the most common clinical diagnosis amongst those that die by suicide but also the most common risk factor for suicidal ideation and behaviour.
- The current provision of PCE-CfD within the NHS is meant to be for 20 sessions (Murphy, 2019). However, this is frequently reduced to 6–12 sessions for those diagnosed with mild-to-moderate depression. These findings demonstrate that the number of sessions might suitably be extended to at least 15 sessions; this study provides preliminary justification for the NHS to extend the offer of PCE-CfD for suicidal clients of up to 20 sessions.

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well-being, regardless of whether suicidal ideation remains part of their authentic experiencing.

Whilst there is extensive research investigating the effectiveness of psychotherapies with suicidal individuals, there remains a lack of empirical research that investigates whether person-centred therapy is effective with this population. Rogers' theory holds that six necessary and sufficient conditions characterise the climate required to facilitate constructive personality change leading to psychological growth (Rogers, 1957b). The six conditions are as follows: there is psychological contact between client and therapist; the client is in a state of incongruence; the therapist is congruent in the relationship with the client; the therapist experiences unconditional positive regard for the client; the therapist experiences empathic understanding for the client; and the client perceives, to a minimal degree, the therapist's unconditional positive regard and empathic understanding. An environment that comprises all six conditions facilitates clients to develop unconditional positive self-regard, helps to align their organismic valuing with the self-concept and dissolves the conditions of worth previously governing them. The individual becomes more congruent, more effective in problem-solving, less vulnerable to threat and less psychologically distressed. Consequently, the individual moves closer towards becoming a fully functioning person (Greenberg & Geller, 2001; Lietaer, 1993; Rogers, 1959). This holistic approach does not specify the direction by which the client moves towards these tendencies and potentials.

Research within the psychotherapy outcome literature has often focussed upon the effectiveness of person-centred therapy in alleviating and reducing symptoms of psychiatric diagnoses, an ideology that does not truly align with person-centred theory of therapy. For instance, a recent meta-analysis of quantitative outcome research surrounding the efficacy of person-centred and experiential therapy was conducted by Elliott et al. (2013). The analysis consisted of 195 outcome studies that investigated the effectiveness of humanistic-experiential psychotherapies, with a total of 14,235 clients. Although humanistic therapies vary in their expression, there is a shared emphasis upon the provision, and curative nature, of a safe, genuine, empathic and unconditionally accepting therapeutic relationship (Cain, 2002). Results confirmed and extended previous meta-analytic findings that humanistic-experiential psychotherapies are effective in alleviating the symptoms of the following: depression, anxiety, psychosis, chronic medical conditions, interpersonal problems, trauma, substance misuse and eating disorders (Elliott et al., 2004; Greenberg et al., 1994). Humanistic-experiential therapies were equally effective in alleviating such diagnostic symptoms when compared with other therapies aligned to the medical model, such as cognitive behavioural therapy (CBT) and psychodynamic therapy. Further to this, a number of studies and meta-analyses found person-centred therapy to be equally effective as CBT, psychodynamic therapy and pharmacotherapy in reducing symptoms of depression, anxiety, postnatal depression, psychosomatic problems and general psychiatric symptoms (Bedi et al., 2000; Bower et al., 2003; Friedli et al., 1997; Gibbard & Hanley, 2008; Gordon & Graham, 1996; Hemmings, 2000; King et al., 2000; Marriott & Kellett, 2009; Rowland et al., 2000; Stiles et al., 2008; Teusch et al., 2001; Ward et al., 2000). Meta-analyses of Rogers' (1957a, 1957b) therapist core conditions found that empathy, congruence and unconditional positive regard are strongly and positively associated with therapeutic outcome (Elliott et al., 2018; Farber et al., 2018; Gelso et al., 2018).

Even though the person-centred approach is not a medical model approach, it is impressive that from a medical model perspective, the evidence is highly supportive for the effectiveness of the Rogerian model of therapy. Nevertheless, such approaches that emphasise the alleviation of symptoms are at odds with the person-centred stance. As person-centred therapy maintains a distinct approach to mental health and well-being, it is vital to conduct research that provides validity to its claim to promote well-being, specifically with psychologically vulnerable and distressed people (Joseph, 2017; Wilkins, 2017). Given the current emphasis upon evidence-based practice within mainstream mental health care, without such research, person-centred therapy will continue to be perceived as an approach that is suitable only for the "worried well" rather than those suffering from serious mental health difficulties (Cooper, 2004; Joseph, 2017; Sommerbeck, 2015).

According to Rogers' (1959) theory of therapy and personality, individuals with greater congruence between self and experience exhibit: greater openness to, and expression of, their organismic experiencing; less defensiveness; psychological adjustment; self-awareness; extensionality; accurate symbolisation of experiences; and effective problem-solving. Accordingly, individuals with such attributes would be able to engage well with the expression and exploration of self-experiences within therapy and, therefore, move towards more optimal psychological adjustment by the end of the therapeutic process. In other words, early levels of authenticity and well-being would be able to predict lower levels of distress later in therapy. However, as it stands, there has been little empirical research investigating this notion and specifically in a sample of suicidal clients. Boyraz et al. (2014) studied the predictive effects of authenticity upon distress over time within a sample of 232 college students. They found that high levels of authenticity at time one were associated with increased life satisfaction and decreased distress after six weeks. Although findings demonstrate the longitudinal predictive nature of authenticity upon distress, the study was not conducted with a clinical sample of suicidal individuals, over the course of person-centred therapy, nor did the study investigate the predictive effects of well-being; thus, research into this area is warranted. With these considerations in mind, this research sought to determine whether person-centred therapy can be effective in not only reducing psychological distress, but also promoting the psychological functioning and well-being in suicidal clients, without targeting suicidal ideation as a symptom specifically in need of treatment.

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2 | METHODS

2.1 | Design

A nonexperimental, naturalistic, longitudinal parametric design was used to investigate whether suicidal clients receiving person-centred therapy experienced statistically significant levels of psychological growth over the course of therapy. The Authenticity Scale (Wood et al., 2008), short Scale of General Well-being (Longo et al., 2018) and Personal Questionnaire (Elliott et al., 1999) were used to measure psychological growth and well-being.

2.2 | Sample

Data were collected from a clinical sample of clients receiving person-centred therapy at a research and counselling clinic in the city of Nottingham. The clinic provides access to free person-centred counselling for individuals aged 18 and above, for up to 40 sessions. The clinic is run by trainee, qualified person-centred therapists and faculty members of the University of Nottingham's Centre for Research in Human Flourishing. Each therapist has demonstrated to a practice panel their competency and adherence using the Person-Centred & Experiential Psychotherapy Scale (PCEPS; Elliott et al., 2011). Each therapist also attends fortnightly supervision with a qualified person-centred therapist.

The research protocol requires all clients to complete a series of outcome measures during intake interview, then prior to sessions 1, 3, 5, 10, 15 and 20, including questionnaires measuring levels of authenticity, well-being and psychological distress. Data were only included for clients who reported suicidal intentions or suicidal ideation as a concern for themselves upon intake. This produced a study sample of $N = 56$ clients. The mean age of clients was 37 years, ranging from 19 to 77 years ($SD = 13.42$, $R = 58$).

The ethnic composition of the clinical sample was 57.1% White British ($n = 32$); 26.8% Other/Unspecified ($n = 15$); 5.4% Black British ($n = 3$); 3.6% British Asian ($n = 2$); and 1.8% White Irish ($n = 1$), Chinese ($n = 1$), Mixed Brazilian ($n = 1$) and Multi-Ethnic ($n = 1$). Alongside suicidal ideation, the composition of self-reported concerns was 78.6% Depression (n = 44); 78.6% Anxiety ($n = 44$); 48.2% Trauma/Abuse ($n = 27$); 46.4% Self-esteem ($n = 26$); 37.5% Interpersonal Relationships ($n = 21$); 19.6% Personality Problems ($n = 11$); 19.6% Bereavement/Loss ($n = 11$); 14.3% Eating Disorder ($n = 8$); 14.3% Work ($n = 8$); 12.5% Physical Problems ($n = 7$); 10.7% Addiction ($n = 6$); 7.1% Living & Welfare ($n = 4$); 7.1% Psychosis ($n = 4$); 5.4% Cognitive & Learning ($n = 3$); and 3.65% Academic ($n = 2$). Clients provided a record of the medication that they were taking: 60.7% were taking antidepressants ($n = 34$), 14.3% were taking antipsychotics ($n = 8$), 3.6% were taking anxiolytics/hypnotics ($n = 2$), 3.6% were taking other/unspecified medication, 1.8% were taking mood stabilisers ($n = 1$) and 1.8% were taking sleep medication ($n = 1$).

2.3 | Measures

The Authenticity Scale (Wood et al., 2008), is a self-report tool used to measure level of client authenticity. The total scale consists of 12 items divided equally into three subscales that represent the tripartite conceptualisation of authenticity, based on person-centred philosophy: self-alienation, accepting external influence and authentic living. Participants were asked to rate and evaluate how well each of the 12 statements described them on a 7-point Likert scale ranging from 1 (Does not describe me at all) to 7 (Describes me very well). The lowest score possible was 12, whereas the highest score possible was 84. Higher scores indicate greater levels of authenticity. The scale previously yielded an acceptable factor structure using exploratory factor analysis, as well as significant discriminant validity from the Big 5 traits of personality and demonstrated reliable psychometric properties with test–retest reliability correlations ranging from 0.78 to 0.91 (Wood et al., 2008). Each subscale has been found to demonstrate high internal consistency: self-alienation ($\alpha = 0.78$); accepting external influence ($\alpha = 0.78$); and authentic living ($\alpha = 0.69$).

The Scale of General Well-being (Longo et al., 2017), a self-report scale measuring psychological growth and well-being through the facets of hedonic (subjective) and eudaimonic (psychological) well-being, is considered a measure of human flourishing and the organismic capacity for growth towards psychological change (ibid.). The short scale (Longo et al., 2018) was used in this study and consists of 14 items—reduced from the original 65-item version and validated in the clinic by Holland et al. (2021). The scale is divided to measure the following indicators of well-being: happiness, involvement, purpose, significance, self-awareness, self-acceptance, self-worth, vitality, calmness, optimism, competence, development, congruence and connection. Participants rated how well items described their levels of well-being on a 5-point Likert scale that ranges from 1 (Not at all true) to 5 (Very true). The lowest total score possible was 14, whereas the highest score possible was 70. Higher scores indicate greater levels of well-being. This short version of the scale was found to confirm the original hierarchical factor structure. It also yielded high test–retest reliability correlations (0.74), internal reliability (McDonald's omega hierarchical coefficients ranging from 0.82 to 0.92) and measurement invariance across age and gender (Longo et al., 2018). Additionally, this scale is considered substantially user-friendly as it is significantly shorter and takes only a small amount of time to complete.

The Personal Questionnaire (PQ; Elliott et al., 1999) measures levels of participants' psychological distress as an individualised and client-generated questionnaire designed to measure changes in psychological difficulties that clients identify as a personal concern for themselves. The PQ items were generated within an initial intake interview where clients were asked to describe the problems that were causing them distress. A list of at least seven (with a maximum of 13) nonredundant items were then generated, which formed the PQ. Participants self-reported the severity of

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TABLE 1 Mean scores, standard deviation and Cronbach's alpha scores

	Mean (*SD*), (α)				
Measure	**T1**	**T2**	**T3**	**T4**	**T5**
Authenticity total	44.82 (3.10), (0.71)	44.82 (3.53), (0.68)	48.18 (3.32), (0.80)	50.91 (3.22), (0.73)	53.27 (3.84), (0.83)
Authentic living	19.09 (6.15), (0.83)	20.24 (6.04), (0.88)	20.17 (5.34), (0.88)	20.40 (5.76), (0.86)	20.85 (5.57), (0.83)
Accepting external influence	16.14 (6.13), (0.86)	16.21 (5.68), (0.81)	16.63 (5.21), (0.82)	15.68 (5.18), (0.84)	15.46 (4.25), (0.58)
Self-alienation	19.70 (6.34), (0.81)	19.29 (6.01), (0.82)	18.51 (6.28), (0.86)	15.88 (6.55), (0.79)	17.92 (7.47), (0.97)
Well-being total	35.55 (2.17), (0.90)	35.64 (3.26), (0.92)	38.91 (3.89), (0.93)	40.36 (4.39), (0.97)	45.00 (4.61), (0.95)
Psychological distress total	37.82 (6.68)	30.91 (5.77)	33.64 (6.08)	31.36 (6.26)	24.55 (4.78)

Note: Authenticity Subscales: Authentic Living (4 items), Accepting External Influence (4 items), Self-Alienation (4 items). T1/T2/T3/T4/T5 = Time 1/Time 2/Time 3/Time 4/Time 5.Time 1/Time 2/Time 3/Time 4/Time 5 = Session 1/Session 5/Session 10/Session 15/Session 20.

Abbreviations: α, Cronbach's alpha; Authenticity, Authenticity Scale (12 items); Psychological Distress, Personal Questionnaire (items vary); SD, Standard Deviation; Well-being, Scale of General Well-being (14 items).

their distress relating to each item within the past seven days on a 7-point Likert scale ranging from 1 (Not at all) to 7 (Maximum possible). Examples of items include "low confidence," "suicidal thoughts" and "low self-esteem." The lowest score possible was 7, whereas the highest score possible was 91. Higher scores indicate greater levels of psychological distress. Conclusions from a recent meta-analysis of five clinical samples provide sound evidence to support the psychometric quality of the PQ (Elliott et al., 2016). Internal consistency between clients varied from 0.70 to 0.80, and test–retest reliability was 0.57. The scale yielded a high sensitivity to detecting change (ibid.). The individualised nature of the PQ allows the capture of the client's unique reality of psychological distress in language that holds most personal meaning to them. Not only does this tackle the shortcoming that nomothetic tools hold in capturing unique realities (Sales & Alves, 2016), but it also fulfils the ethical commitment person-centred therapists make to fostering client autonomy (Rogers, 1951; Schmid, 2005). Additionally, it aligns well with the person-centred emphasis upon each human "as a *unique* entity" (Cain, 2002, p. 5).

2.4 | Procedure

The clinic data were filtered to contain solely those records that fell into the eligibility criteria of clients that considered themselves at elevated risk of suicide. Records containing errors or incomplete data were removed. These data were collated into an encrypted Excel spreadsheet and transferred into SPSS v25 software, wherein all statistical analyses were performed. Before commencing analyses, items that required reverse coding were processed within SPSS. This constituted two subscales of the Authenticity Scale (Wood et al., 2008); the Accepting External Influence subscale (items 3, 4, 5 and 6), as well as the Self-alienation subscale (items 2, 7, 10 and 12). Following this, total scale scores were calculated for each of the three research measures across Sessions 1 (T1), 5 (T2), 10 (T3), 15 (T4) and 20 (T5).

2.5 | Data analysis

To determine statistically significant changes in mean scores of authenticity, well-being and psychological distress within clients across five time points of person-centred therapy, one-way repeated measures analysis of variance (ANOVA) was undertaken. To test that the data for the Authenticity Scale were suitable for use in ANOVA, Mauchly's Test of Sphericity was carried out. Mauchly's Test indicated that the assumption of sphericity had not been violated, $\chi^2(9) = 12.011$, $p = 0.221$. The test was repeated for the data for short Scales of General Well-being. Mauchly's Test of Sphericity was carried out and indicated that the assumption of sphericity had not been violated, $\chi^2(9) = 13.346$, $p = 0.153$. To test that the data for the Personal Questionnaire were suitable for use in ANOVA, Mauchly's Test of Sphericity was carried out for a third time. Mauchly's Test indicated that the assumption of sphericity had been violated, $\chi^2(9) = 18.721$, $p = 0.030$, meaning the Huynh–Feldt correction was applied, $\varepsilon = 0.848$.

The repeated measures test yields an "omnibus" statistic; thus, although we can determine whether means significantly differed from one another, the repeated measures ANOVA cannot determine which specific time points were statistically different. To address this, post hoc tests and paired samples t-tests were carried out, allowing comparisons of means over time whilst maintaining the Type 1 error rate at the priori level (Howell, 2002).

In order to determine the strength and direction of the relationship between authenticity, well-being and psychological distress, Pearson's correlation tests were undertaken. Statistical significance is reported with $r = 0.10$ representing a small effect size, $r = 0.30$ representing a medium effect size and $r = 0.50$ representing a large effect size (Cohen, 2013). Lastly, the relative degree to which associated scores of authenticity, well-being and psychological distress predict one another was investigated using simple linear regression analyses. Linear regressions were preferred for their ease of reporting and interpretation whilst maintaining the underlying assumption of linear relations amongst variables.

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3 | RESULTS

Table 1 displays the descriptive statistics for all three of the measures used throughout therapy at T1 (Session 1), T2 (Session 5), T3 (Session 10), T4 (Session 15) and T5 (Session 20). There is an obvious positive trend for the mean scores of authenticity and well-being, as well as a negative trend for the mean scores of psychological distress. The table also displays Cronbach's alpha scale reliability test scores, which were conducted to determine consistency of the underlying constructs measured (Field, 2018). Results revealed high levels of reliability for all scales, as most values exceeded the threshold of $\alpha = 0.70$ (Kline, 1999), with the exceptions being the Accepting External Influence subscale at T5 and the total sco for the Authenticity Scale at T2.

3.1 | One-way repeated measures ANOVA

A one-way repeated measures ANOVA indicated changes in the mean scores of the Authenticity Scale were significant between time points [$F(4, 40) = 3.562$, $p < 0.014$]. Post hoc tests using the Bonferroni correction revealed that there was an increase in scores on the Authenticity Scale amongst suicidal clients between T1 and T5, which was not statistically significant ($p = 0.328$). However, there was a statistically significant increase in scores on the Authenticity Scale between T2 and T5 ($p = 0.034$), by an average of 8.46 points. Results also indicated that mean scores of the Scale of General Well-being changed significantly between time points [$F(4, 44) = 4.024$, $p < 0.007$]. Post hoc tests using the Bonferroni correction revealed that there was an increase in levels of well-being amongst suicidal clients between T1 and T5, which was not statistically significant ($p = 0.202$). However, there was a statistically significant increase in scores of the Scale of General Well-being between T2 and T5 ($p = 0.002$), by an average of 8.92 points. Lastly, results also indicated that mean psychological distress, as measured by the PQ, changed significantly between time points [$F(2.553, 28.083) = 5.228$, $p < 0.008$]. Post hoc tests using the Bonferroni correction revealed that there was a decrease in levels of psychological distress amongst suicidal clients between T1 and T5, which was not statistically significant ($p = 0.057$). However, a statistically significant decrease in PQ scores was found between T2 and T5 ($p = 0.015$), by an average of 6.25 points.

3.2 | Paired samples *t*-tests

Results from the t-test revealed that there were no statistically significant changes in scores for the Authenticity Scale, Scale of General Well-being or PQ between T1 and T2 or T1 and T3 (Tables S1 and S2).

However, there was a statistically significant increase in levels of authenticity (Table S3) between T1 ($M = 47.16$, $SD = 8.68$) and T4 ($M = 51.96$, $SD = 10.78$), $t(24) = -2.893$, $p = 0.008$. Moreover, there was a statistically significant improvement in levels of well-being between T1 ($M = 32.17$, $SD = 9.40$) and T4 ($M = 38.92$, $SD = 15.00$), $t(23) = -2.871$, $p = 0.009$. Lastly, there was a statistically significant decrease in levels of psychological distress between T1 ($M = 40.29$, $SD = 19.24$) and T4 ($M = 35.38$, $SD = 19.76$), $t(23) = 2.335$, $p = 0.029$.

Results from the t-test reveal that there was a statistically significant increase in levels of authenticity (Table S4) between T1 ($M = 46.00$, $SD = 9.87$) and T5 ($M = 54.23$, $SD = 12.35$), $t(12) = -2.789$, $p = 0.016$. There was also a statistically significant improvement in levels of well-being between T1 ($M = 34.92$, $SD = 6.83$) and T5 ($M = 44.08$, $SD = 14.15$), $t(12) = -2.997$, $p = 0.011$. Lastly, results show that there was a statistically significant decrease in levels of psychological distress between T1 ($M = 38.46$, $SD = 20.31$) and T5 ($M = 26.08$, $SD = 15.03$), $t(12) = 3.557$, $p = 0.004$.

4 | NATURE OF RELATIONSHIPS BETWEEN AUTHENTICITY, WELL-BEING AND PSYCHOLOGICAL DISTRESS

4.1 | Correlational analysis

Although Table S5 shows there were several significant associations between variables over the course of therapy, results from T1 and T4 will be reported as the ANOVA, and t-tests found that significant changes occur by T4. In regard to assessing the magnitude of the zero-order correlations between each measure, statistical significance is reported with $r = 0.10$ representing a small effect size, $r = 0.30$ representing a medium effect size and $r = 0.50$ representing a large effect size (Cohen, 2013).

4.2 | Authenticity and psychological distress

There was a significant negative association between authenticity at T1 and psychological distress at T1, $r(56) = -0.33$, $p = 0.014$. There was also a significant negative association between authenticity at T4 and psychological distress at T4, $r(24) = -0.47$, $p = 0.045$. Lastly, there was a significant negative association between authenticity at T1 and psychological distress at T4, $r(24) = -0.44$, $p = 0.033$.

4.3 | Well-being and psychological distress

There was a significant negative association between well-being at T1 and psychological distress at T1, $r(56) = -0.41$, $p = 0.002$. Additionally, a significant negative association was found between well-being at T4 and psychological distress at T4, $r(24) = -0.65$, $p = 0.001$. There was also a significant negative association between well-being at T1 and psychological distress at T4, $r(24) = -0.61$, $p < 0.001$.

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4.4 | Authenticity and well-being

There was a significant positive association between authenticity at T1 and well-being at T1, $r(56) = 0.34$, $p = 0.011$. Moreover, there was a significant positive association between authenticity at T4 and well-being at T4, $r(24) = 0.53$, $p = 0.008$. There was no such significant positive association between authenticity at T1 and well-being at T4. Interestingly, significant positive associations emerged between well-being at T1 and authenticity at T4, $r(25) = 0.68$, $p < 0.001$.

4.5 | Simple linear regression

A simple linear regression revealed that, for authenticity, a significant regression was found $F(1, 22) = 5.21$, $p = 0.033$, with an R^2 of 0.16. For well-being, a significant regression was also found, $F(1, 22) = 12.87$, $p = 0.002$, with an R^2 of 0.34. A simple linear regression was carried out to examine whether well-being at T1 could predict levels of authenticity at T4, as a significant association was found within the correlation analysis. A significant regression was found $F(1, 23) = 19.53$, $p < 0.001$, with an R^2 of 0.46.

5 | DISCUSSION

This study set out to investigate whether suicidal clients receiving person-centred therapy demonstrated greater authenticity, well-being and psychological distress over time. Descriptive statistics and ANOVA with t-test analyses revealed that suicidal clients significantly increased in authenticity and well-being, as well as decreased in psychological distress over the course of person-centred therapy. A key finding from post hoc tests was that no statistically significant changes within authenticity, well-being and psychological distress were observed until at least five sessions of therapy had taken place. At least 15 sessions of therapy were required to see statistically significant.

Minimum authenticity scores decreased by three points from T1 to T5, and minimum well-being decreased by a noteworthy amount of nine points. Although this does not align with the expected trend, further investigations and follow-up would provide a clearer understanding of the rationale behind this trend.

These results support the theory of the person-centred approach to therapy by demonstrating that, in the context of person-centred therapy, clients are facilitated to move towards greater congruence and psychological adjustment (Rogers, 1957b/1963). This research rejects acclamations that person-centred therapy is ineffective for supporting clients experiencing suicidal ideation or is a therapy merely for the "worried well" (Wilkins, 2003). Furthermore, such findings have extended prior knowledge in that they reveal that it takes, on average, at least five sessions of therapy to begin to see psychological growth, as well as 15 sessions of therapy to see changes in statistical significance within this particular sample of clients. For instance, as Rogers (1959) hypothesised in his theory of therapy, clients may enter therapy with little or no self-awareness; little positive self-regard; a degree of incongruence, with intensionality and defensiveness; conditions of worth; and experiences that may be distorted or denied to the awareness. However, as the individual gradually begins to perceive the therapist's unconditional positive regard and empathy, and begins to build a sense of safety and trust with the therapist, the client gradually develops unconditional positive self-regard and therapeutic change begins to occur (Mearns & Thorne, 2000). There is a gradual loosening of constructs and breakdown of one's conditions of worth; a gradual movement from fixity to changingness; a gradual increase in the expression and flow of thoughts and feelings; gradual exploration of one's incongruences; and reorganisation of the self-concept to assimilate previously denied experiences (Rogers, 1959/1963). As shown in this study, all such developments are a steady and timely process; perhaps particularly, for clients that are so severely distressed that they are experiencing a desire to end their own life.

This study also attempted to understand the associations between authenticity, well-being and psychological distress within a suicidal sample. Correlation analyses revealed statistically significant negative relationships between authenticity and well-being and psychological distress both at T1 and at T4. In addition to this, statistically significant negative associations were found between authenticity and well-being at T1 and psychological distress at T4. Lastly, positive associations were found between authenticity and well-being at both T1 and T4. Such findings support the notion that authenticity and well-being play an important role in psychological growth and functioning (Joseph & Linley, 2005). Moreover, the findings extend this and suggest that this notion holds true within a sample of suicidal clients.

Interestingly, associations between each of the variables started at a medium effect size in early stages of therapy and increased to even larger effect sizes during later stages of therapy. Further investigations to determine why authenticity and well-being become increasingly associated over time would provide further insights into understanding the nature of the association between authenticity and well-being. For example, in the light of the positive associations between authenticity and well-being, it is possible there may be some overlap between the two concepts, explaining why the subjective experience of well-being and authenticity increases over time in therapy. This seems logical as well-being is said to reflect indicators such as self-awareness, congruence and self-acceptance (Longo et al., 2017), which are synonymous with Rogers' concept of congruence and the fully functioning person (Rogers, 1963).

Lastly, this study aimed at determining the predictive nature of authenticity and well-being. Simple linear regression analyses revealed that authenticity and well-being significantly predicted later levels of psychological distress in suicidal clients. Such findings support the notion that the greater degree of congruence and well-being experienced, the less distressed individuals are likely

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to be. At later stages of therapy, as the theory suggests, clients were able to engage well with the exploration of their inner experiencing throughout therapy. This can be explained by the notion that the more authentic and closer to optimal psychological adjustment, the less psychological tension is experienced, meaning people are: more effective in problem-solving; more extensional and expressive in relation to their inner experiencing; less vulnerable to threat; and better able to accurately symbolise their organismic needs within their awareness (Rogers, 1959).

Unexpectedly, authenticity was not found to significantly correlate with, or predict, later levels of well-being. This could potentially be attributed to the small sample size of the study. Interestingly, well-being was found to significantly correlate with, and predict, later levels of authenticity, which was not originally predicted. This would suggest, theoretically, that facets of well-being, such as happiness, vitality, calmness, optimism, involvement, self-awareness, self-acceptance, self-worth, competence, development, purpose, significance and connection (Longo et al., 2017) impact the degree to which one lives in accordance with their organismic experiencing. This suggests that therapy that works towards increasing well-being might lead to greater authenticity, rather than the other way around. Both further quantitative and qualitative research would enable a more accurate understanding regarding the nature of the causal impact of well-being upon authenticity.

5.1 | Implications for practice

In the National Health Service (NHS), person-centred experiential counselling for depression (PCE-CfD) is one of the currently available therapies for people experiencing depression. Depression is not only the most common clinical diagnosis amongst those that die by suicide, but also the most common risk factor for suicidal ideation and behaviour. This notion has been extensively and consistently demonstrated by a large body of research and systematic reviews (Cai et al., 2021; Chang et al., 2009; da Silva et al., 2006; Diaconu & Turecki, 2007; Dong et al., 2018; Harwood et al., 2001; Hawton et al., 2013; Hawton & van Heeringen, 2009; Henriksson et al., 1993; Isometsa et al., 1994; Kessler et al., 1999; Lee et al., 2007; Lonnqvist, 2000; McGirr et al., 2007; Nock et al., 2010; Séguin et al., 2007; Tegegne et al., 2022). Within the NHS, in accordance with such findings, most suicidal clients that are diagnosed with depression are often referred for counselling or psychotherapy.

The current provision of PCE-CfD within the NHS is meant to be for 20 sessions (Murphy, 2019). This approach has been evidenced in a randomised controlled trial (Barkham et al., 2021). However, this is frequently reduced to 6–12 sessions for those diagnosed with mild-to-moderate depression. The present findings demonstrate that the number of sessions might suitably be extended to at least 15 sessions, as it is only after this point that psychological growth is likely to be facilitated within suicidal clients; this study provides preliminary justification for the NHS to extend the offer of PCE-CfD for suicidal clients to up to 20 sessions.

5.2 | Limitations

A limitation of the current study is that of attrition; the loss of participants over the course of the research process. There were only 13 clients, of the 56 clients at the start of therapy, who completed the full course of 20 sessions. The reasons for this attrition were largely unknown. Within longitudinal studies of psychotherapy, attrition can often be attributed to a number of reasons, including: physical illness, relocation, practical barriers to attending, overwhelm of distress or even death (Little, 1995; Weston, 2005; Young et al., 2006). However, it has also been shown that some clients end therapy having reached a good enough level of change (Reese et al., 2011). Future studies may consider gaining consent to make follow-up contact with participants to understand the reasons for attrition; this would allow for a more accurate analysis of data.

6 | CONCLUSION

In the light of the gaps that were identified within the current body of psychotherapy literature, this study aimed at determining whether person-centred therapy was effective in promoting authenticity, well-being and reduced psychological distress within clients experiencing suicidal ideation. The study also aimed at examining the relationships between, and predictive nature of, authenticity and well-being upon distress. Findings supported the notion that clients receiving person-centred therapy experience improvements in authenticity, well-being and psychological distress; thus, indicating that person-centred therapy is not merely a therapy for the "worried well." The study also supports the hypothesis that authenticity and well-being are negatively associated with, and significant predictors of, psychological distress. It is hoped that future research will further develop our understanding of person-centred practice with those experiencing suicidal ideation, as well as increase the degree of credibility that person-centred therapy receives within mainstream settings for working with this group of clients.

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SUPPORTING INFORMATION

Additional supporting information can be found online in the Supporting Information section at the end of this article.

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